



Policy and Procedure Manual

Volume 1, Chapter 16

Nutrition Education

Washington WIC Program

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Nutrition Education

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Section 1: Introduction

Introduction:

Nutrition services, including nutrition education, are the core and heart of WIC. Nutrition education is the primary benefit that distinguishes WIC from other nutrition assistance programs as a premier public health program. Staff use information about the participant's nutrition and health status gathered during the nutrition assessment to personalize nutrition education. Providing quarterly participant-centered nutrition education is key to helping families achieve positive health outcomes.

A. The Goals of the Nutrition Assessment

The nutrition assessment:

- Collects and incorporates the participant's nutrition and health information.
- Assesses for the participant's nutrition and breastfeeding status.
- Identifies the participant's strengths and needs.
- Is used to determine eligibility and personalize WIC nutrition services.
- Uses the Value Enhanced Nutrition Assessment (VENA) approach which is participant-centered and health outcome-based.

B. Definition and Goals of Nutrition Education

Definition:

Nutrition education means offering counseling and information to help the participant improve their health status. Nutrition education emphasizes the relationship between nutrition, physical activity, and health, and aligns with the participant's personal and cultural preferences, interests and needs.

Basic nutrition education: Nutrition education the Competent Professional Authority (CPA) provides as part of the certification appointment.

- The basic nutrition education occurs at the Initial Certification, Presume Eligible Complete Assessment, and the Subsequent Certification.

Second nutrition education: In addition to the basic nutrition education, staff provide additional nutrition education approximately once every three months (see the [Table of Nutrition Education Contacts by Participant Category](#) on the web) during the certification period at the following appointments:

- a. The Infant, Child, or Breastfeeding Mid-Certification Health Assessment.

Note: Staff can provide these appointments in person or remotely.

- 1) Staff complete the Mid-Certification Health Assessment approximately halfway through the participant's certification period.
 - 2) Staff perform or obtain anthropometric and hematology values.
 - 3) Follow-up on previous risks and concerns and document new risks as appropriate.
- See the ["Nutrition Education at the Mid-Certification Health Assessment"](#) policy for more information.

- b. The Individual, RD, Online, or Group Class nutrition education contact.

Note: Staff can provide these appointments in person or remotely. Staff can also offer individual nutrition education online using WIC Health.

- 1) Individual: A one-on-one participant-centered conversation about the participant's needs and concerns. See the [Second Nutrition Education Contact - Individual](#) policy.
- 2) RD: Registered Dietitian. The appointment for a high risk contact with a nutritionist is called a RD appointment in Cascades. This appointment is completed by a WIC Nutritionist who provides nutrition counseling and writes a high risk care plan for participants identified as high risk. See the [Second Nutrition Education - High Risk with a WIC Nutritionist](#) policy.

Note: The WIC Nutritionist may be a Registered Dietitian or a person with a master's degree in the field of nutrition from an accredited college or university.

- 3) Online: An interactive online lesson completed independently by the participant on wichealth.org. [See the Online Nutrition Education Contacts](#) policy.
 - The WIC Health online lesson is relevant to the category, needs, and preferences of the participant or parent guardian.
- 4) Group Class: A facilitated group discussion with two or more participants. See the [Second Nutrition Education - Group Class](#) policy.

- The group class is relevant to the category, needs, and preferences of the participant or parent guardian.
- Staff provide an opportunity for attendees to talk and learn with each other.

Goals:

The goals of nutrition education are to:

1. Support existing positive nutrition habits or health practices.
2. Inspire and facilitate the participant achieving dietary, physical activity, and health habits resulting in improved nutritional status and the prevention of nutrition-related problems.

By offering counseling that explores participants' thoughts and feelings and guides them to their own goals, participants can move towards health behaviors that are truly meaningful and relevant for them and their families.

C. Definition and Goals of Participant Centered Counseling

Definition:

Participant centered – a collaboration where staff connect through careful listening and empathy with the participant on topics relevant to the participant. Staff consider the participant's capabilities, strengths, values, cultural practices and customs, environmental factors, family influences and individual needs and tailors the conversation to what is most important to the participant. Staff use open-ended questions, affirmations, reflections, and summaries (OARS) to individualize nutrition education and guide the participant towards meaningful behavior change.

Goals:

The goals of participant centered counseling are to:

1. Support the participant or parent guardian to achieve their goals or to maintain existing positive health practices.
2. Guide the participant or parent guardian towards their own solutions, ideas, or behavior change goals.
 - Emotions, feelings, hopes, and dreams drive people toward behavior change more than logic and facts.

Note: [WIC Connects](#) is Washington WIC's participant-centered approach to providing WIC services. WIC Connects is about improving services for participants and engaging staff in more meaningful and effective WIC work.

D. Value Enhanced Nutrition Assessment (VENA)

Definition:

[VENA](#) is a participant-centered, health outcome-based approach to a WIC nutrition assessment, which is a required and essential part of the WIC program.

Goals:

The goals of VENA are to determine eligibility by identifying nutrition risks using a systematic approach and assessing information elicited from the participant. At the same time allowing staff to engage the participant in dialogue about their needs and goals for healthy behavior.

Section 2: Requirements for Nutrition Education

POLICY: Required Nutrition Education Contacts

Staff must offer each participant the required number of nutrition education contacts during the certification period. WIC participants receive a minimum of four nutrition education contacts during a 1-year certification period. The process repeats with the subsequent certification.

- See the [Table of Nutrition Education Contacts by Participant Category](#) on the web.

Nutrition Education is offered at the following appointments:

1. The Initial and Subsequent Certification, and the Presume Eligible Complete Assessment.
2. The Infant, Child, or Breastfeeding Mid-Certification Health Assessment.
3. The second nutrition education contact – Individual, RD, Online, or Group Class.

PROCEDURE:

Staff:

- A. Assure each participant receives the number of required nutrition education contacts.
- B. Assess the needs of the participant or the parent guardian and choose the most effective method of offering nutrition education, for example in person, remote, or online.
- C. Schedule the appointment type for the determined method of delivery.
 - Make at least one attempt to reschedule the appointment and document in a consistent location in the participant's file.
 - Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete a nutrition education appointment.

POLICY: Nutrition Education at the Initial and Subsequent Certification and Presume Eligible Complete Assessment

Staff must:

1. Provide nutrition education at the Initial and Subsequent Certification and Presume Eligible Complete Assessment.

Note: Staff can provide these appointments in person or remotely.

2. Review the nutrition risks and concerns identified from the assessment.
3. Have a [participant-centered](#) conversation about the participant's nutrition, breastfeeding, physical activity, and health related needs, interests, and goal(s).
 - Explore information related to participant's nutrition risks and other topics relevant or of interest to the participant/parent guardian as the menu of topics to choose from.
 - Assess every pregnant participant's thoughts about infant feeding and providing their milk at these appointments and offer information based on the participant's needs.
 - When the parent or guardian doesn't express a topic or area of interest, offer nutrition education based on topics relevant to the participant's age, risks and category or common areas of concern for participants of a similar age, category, or situation.
4. Document a note in the participant's Individual Care Plan that includes:
 - a. The appointment type as the title.
 - b. The participant/parent guardian's thoughts and feelings about the topic(s).
 - c. Information offered/shared/discussed about the topic(s).
 - d. The participant's goal, if the participant is ready to set a goal or document that the participant wasn't ready to set a goal.
 - e. Additional information for future support and follow up.

5. Document the nutrition education topic(s) discussed and mark as “Complete” in the Care Plan – Nutrition Education.

PROCEDURE:

Staff:

- A. Review information from the assessment:
 1. Assess the participant’s nutrition, breastfeeding, physical activity, and health related needs and interests.
 2. Analyze and prioritize topics for discussion.
- B. Facilitate a [participant centered](#) conversation with the participant or parent guardian.
 1. Explore information related to participant’s nutrition risks and other topics relevant or of interest to the participant/parent guardian.
 - Use the nutrition risks and needs identified in the assessment as the menu of topics to choose from.
 - Provide drug and other harmful substance abuse information to all participants, parent guardians, and caretakers at the initial certification.
 - Assess every pregnant participant’s thoughts about infant feeding and providing their milk at these appointments and offer information based on participant’s needs.
 - When the parent or guardian doesn’t express a topic or area of interest, offer nutrition education based on topics relevant to the participant’s age, risks and category or common areas of concern for participants of a similar age, category, or situation.
 2. Have a goal setting conversation to explore goals or next steps.
 3. Summarize next steps and affirm plans for behavior change as appropriate.
- C. Document a note in the participant’s Individual Care Plan as per policy requirements.

Note: Staff have the option to document goals on the Maintain Goals screen in addition to the requirement to document goals in the Individual Care Plan.

- D. Document the nutrition education topic(s) discussed and mark as “Complete” in the Care Plan – Nutrition Education.

Information:

Stress the positive, long-term benefits of nutrition education and encourage the participant to attend and participate in nutrition education activities.

Staff can share this information as part of the introduction to WIC or when scheduling the next appointment.

POLICY: Nutrition Education at the Mid-Certification Health Assessment

Staff must:

1. Provide nutrition education at the Mid-Certification Health Assessment.
 - See [Volume 1, Chapter 18 - Certification](#) for information about Mid-Certification Health Assessment requirements.

Note: Staff can provide these appointments in person or remotely.
2. Follow up on nutrition risks and concerns identified at the certification and/or the second nutrition education contact.
3. Have a [participant centered](#) conversation about the participant's needs and concerns.
 - When the parent or guardian doesn't express a topic or area of interest, offer nutrition education based on topics relevant to the participant's age, risks and category or common areas of concern for participants of a similar age, category, or situation.
4. Follow-up with the participant about their goal(s) as needed.
5. Document a note in the participant's Individual Care Plan that includes:
 - a. The appointment type as the title.
 - b. The participant/parent guardian's thoughts and feelings about the topic(s).
 - c. Information offered/shared/discussed about the topic(s).
 - d. Update information about the participant goal(s) as needed.
 - e. Additional information for future support and follow up.
6. Select "Mid-Cert HA Completed" and document the nutrition education topic(s) discussed as "Complete" in the Care Plan – Nutrition Education.
7. Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete a mid-certification health appointment.
 - Make one attempt to reschedule a nutrition education contact and document it in a consistent location in the participant's file.

PROCEDURE:

Staff:

- A. Follow up on any ongoing nutrition risks and concerns identified at previous appointments.
 - Analyze and prioritize topics for discussion.
- B. Facilitate a [participant centered](#) conversation with the participant or parent guardian.
 1. Explore information related to participant's nutrition risks and other topics relevant or of interest to the participant or parent guardian.
 - Use the nutrition risks and needs identified in the assessment as the menu of topics to choose from.
 - When the parent or guardian doesn't express a topic or area of interest, offer nutrition education based on topics relevant to the participant's age, risks and category or common areas of concern for participants of a similar age, category, or situation.
 2. Summarize next steps and affirm plans for behavior change as appropriate.
- C. Document a note in the participant's Individual Care Plan as per policy requirements.

Note: Participants don't have to set a goal at the Mid-Certification Health Assessment; however, follow up and support for previously set goals can help move participants towards positive behavior changes.
- D. Document the nutrition education topic(s) discussed and mark as "Complete" in the Care Plan – Nutrition Education.
- E. Select the "Mid-Cert HA Completed" topic in addition to the nutrition education topic(s) discussed.
- F. Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete a mid-certification health appointment.
 1. Make one attempt to reschedule the mid-certification health assessment.
 2. Document the attempt to reschedule in the participant's file.

- a. Staff document in a consistent location in the participant's file according to the clinic's practice.

POLICY: Second Nutrition Education – Individual

Staff must:

1. Provide a second nutrition education contact (Individual, Group Class, or Online) to non-high risk participants based on their category, needs, and preferences.

Note: The individual and group nutrition education contact may be done in-person or remotely.

- See the [Table of Nutrition Education Contacts by Participant Category](#) on the web.
 - For high risk participants see the [Second Nutrition Education – High Risk with a WIC Nutritionist](#).
2. Follow up on nutrition risks and concerns identified at the certification.
 3. Have a [participant centered](#) conversation about the participant’s needs and concerns.
 - When the parent or guardian doesn’t express a topic or area of interest, offer nutrition education based on topics relevant to the participant’s age, risks and category or common areas of concern for participants of a similar age, category, or situation.
 4. Follow-up with the participant about their goal(s) if a goal was set.
 5. Document a note in the participant’s Individual Care Plan that includes:
 - a. The appointment type as the title.
 - b. The participant or parent guardian’s thoughts and feelings about the topic(s).
 - c. Information offered/shared/discussed about the topic(s).
 - d. Updated information about the participant goal(s) if a goal was set.
 - e. Additional information for future support and follow up.
 6. Document the nutrition education topic(s) discussed and mark as “Complete” in the Care Plan – Nutrition Education.

7. Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete a second nutrition education appointment.
 - Make one attempt to reschedule a nutrition education contact and document it in a consistent location in the participant's file.

PROCEDURE:

Staff:

- A. Facilitate a [participant centered](#) discussion with the participant or parent guardian to either:
 1. Check-in on previously identified topics, goal(s), or next steps.
 - Depending on the status of the participant's progress towards meeting their goal:
 - Affirm progress toward their goal or
 - Explore new ways to support progress toward their goal.
 - Reflect, affirm, and/or summarize the outcome.
 2. Explore a new topic(s) that is:
 - Of the participant's choice or relevant to the needs and interests of the participant or parent guardian.
 - When the parent or guardian doesn't express a topic or area of interest, offer nutrition education based on topics relevant to the participant's age, risks and category or common areas of concern for participants of a similar age, category, or situation.
 - Facilitate a goal setting conversation about the new topic, if appropriate, however, it's not required.
- B. Document a note in the participant's Individual Care Plan as per policy requirements.

Note: Staff have the option to document goals on the Maintain Goals screen in addition to the requirement to document goals in the Individual Care Plan.

- C. Document the nutrition education topic(s) discussed and mark as “Complete” in the Care Plan – Nutrition Education.
- D. Don’t withhold food benefits or limit to monthly issuance when the participant doesn’t complete a second nutrition education appointment.
 - 1. Make one attempt to reschedule a nutrition education contact:
 - 2. Document the attempt to reschedule in the participant’s file.
 - a. Staff document in a consistent location in the participant’s file according to the clinic’s practice.

POLICY: Second Nutrition Education – Group Class

Staff:

1. Have the option to offer a second nutrition education contact as a Group Class to non-high risk participants based on their category, needs, and preferences.

Note: The group nutrition education contact may be done in-person or remotely.

- A Group Class can't be used to meet the high risk care plan requirement for participants designated as high risk. However, it could be offered as a second nutrition education contact after a high risk care plan is completed.
 - For high risk participants see the [Second Nutrition Education – High Risk with a WIC Nutritionist](#).
 - See the [Table of Nutrition Education Contacts by Participant Category](#) on the web.
2. Use an interactive method when providing a Group Class to engage participants in discussion and to offer an opportunity for participants to learn from each other.
 - For example, a [Facilitated Group Discussion](#) format.
 - The Group Class can be remote or in-person.
 3. Document class attendance in the clinic Master Calendar.
 - Cascades documents the group class topic in the participant's file on the Nutrition Education screen.
 4. Keep the Group Class lesson plan or Facilitated Discussion Guide on file for 4 years.
 5. Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete a group nutrition education class.
 - Make one attempt to reschedule a nutrition education contact and document it in a consistent location in the participant's file.

PROCEDURE:

Staff:

- A. When scheduling, assure the Group Class is relevant to the category, needs, and preferences of the participant or parent guardian.
 - If the Group Class topic isn't relevant for a participant in the household or the participant is high risk, schedule an individual nutrition education contact or a high risk contact for the participant.
- B. Facilitate a group discussion where participants or parent guardians have the opportunity to talk and learn from each other.
 1. Explore the topic while considering the interests of the group.
 2. Assure participants receive accurate and relevant information through a facilitated group discussion.
 3. Affirm any next steps shared by participants during the facilitated group discussion.
- C. Document class attendance in the clinic Master Calendar.
 - Documenting class attendance marks the topic for the group class as "Complete" for each participant in the family when the topic applies to their category.
- D. Keep the Group Class lesson plan or Facilitated Discussion Guide on file for 4 years.
- E. Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete a group nutrition education class.
 1. Make one attempt to reschedule a nutrition education contact:
 2. Document the attempt to reschedule in the participant's file.
 - a. Staff document in a consistent location in the participant's file according to the clinic's practice.

Information:

Facilitated group education is a group of two or more participants with similar interests and needs coming together to talk and learn with each other. During the Facilitated Group Education staff use a variety of interactive methods, such as [Talk, Listen, Connect](#) (TLC), to engage participants in nutrition, breastfeeding, healthy living, and active living topics relative to their category, needs, and interests. A facilitated group discussion can be in person or remote via video chat.

POLICY: Second Nutrition Education – High Risk with a WIC Nutritionist

The WIC Nutritionist must:

1. Provide a high risk contact and write a High Risk Care Plan (HRCP) for participants identified as high risk based on Washington WIC 's nutrition high risk criteria or as Professional Discretion High Risk.
 - The Competent Professional Authority (CPA) can make a participant high risk by participant request, or professional discretion at the certification appointment. See the [Staff Tool: Professional Discretion High Risk \(PDHR\)](#).
2. See the participant one time per certification unless additional follow up is needed and determined by the nutritionist, or a new high risk factor is identified later in the eligibility period.
 - Once the HRCP is completed, staff may offer an individual, online, or group second nutrition education contact unless additional follow-up is needed or a new high risk factor is identified later.
 - The participant isn't required to see the nutritionist for the HRCP when a high risk participant becomes non-high risk at a later visit. Staff must write a note in the participant's file on the Individual Care Plan Summary when the participant becomes non-high risk.
 - See the [Table of Nutrition Education Contacts by Participant Category](#) on the web.
3. Offer nutrition topics guided by both the identified high risk factors and the participant's or parent guardian's interests or needs.
4. Have a [participant centered](#) conversation about the nutrition topic(s) identified.
5. Facilitate a goal setting conversation related to the participant's risks, interests, or needs.
6. Document a High Risk Care Plan (HRCP) in the Individual Care Plan which includes:
 - a. The title: HRCP.
 - b. The participant/parent guardian's thoughts and feelings about the topic(s).

- c. Information offered/shared/discussed about the topic(s).
 - d. The participant's or parent guardian's goal, if they're ready to set a goal or document that they weren't ready to set a goal.
 - e. The plan for future contact or additional information as determined by the nutritionist.
- 7. Select "HRCP Completed" and document the nutrition education topic(s) discussed as "Complete" in the Care Plan – Nutrition Education.
 - 8. Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete a high risk nutrition education appointment.
 - Make one attempt to reschedule a nutrition education contact and document it in a consistent location in the participant's file.

The WIC Nutritionist can accept a high risk care plan written by an outside registered dietitian. In this case the nutritionist must:

- 1. Obtain the participant's or parent guardian's signature on a Release of Information form or confirm the outside agency has a current Data Sharing Agreement with the WIC agency prior to contacting the outside dietitian.
- 2. Scan the written care plan into Cascades and document a note in the Individual Care Plan to direct staff where to find the HRCP.

PROCEDURE:

The Competent Professional Authority (CPA):

- A. Assess if the participant is high risk.
 - 1. The participant has a high risk factor assigned in Cascades.
 - 2. The CPA determines the participant should see the nutritionist and makes the participant high risk by professional discretion at the initial or subsequent certification appointment.
 - Select the High Risk "Yes" radio button on the Certification Summary screen **before** pressing the Certify button.

3. The participant becomes high risk or has an additional high risk factor documented during the certification period.
 - Schedule the participant with the nutritionist when the participant becomes high risk and there are more than 60 days left in the eligibility period.
 - It's not required to schedule participants with less than 60 days in their eligibility; however, staff may choose to schedule the participant for a high risk appointment when possible.
 4. The participant doesn't have a high risk factor assigned but the participant requests to see the nutritionist.
- B. Assure high risk participants receive a high risk second contact as per policy.
- C. Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete a high risk nutrition education appointment.
1. Make one attempt to reschedule a nutrition education contact:
 2. Document the attempt to reschedule in the participant's file.
 - a. Staff document in a consistent location in the participant's file according to the clinic's practice.

The WIC Nutritionist:

- A. Has a [participant centered](#) conversation about the high risk nutrition topic(s) identified.
- B. Facilitates a goal setting conversation related to the participant's risks, interests, or needs.
- C. Documents a High Risk Care Plan (HRCP) in the Individual Care Plan as per policy requirements.
 - Use a standardized individual care plan documentation method, such as:
 - Subjective, Objective, Assessment, Plan Intervention, Evaluation (SOAPIE),
 - Assessment, Diagnoses, Intervention, Monitoring & Evaluation (ADIME)

Note: The nutritionist has the option to document goals on the Maintain Goals screen in addition to the requirement to document goals in the Individual Care Plan.

- D. Selects “HRCP Completed” and document the nutrition education topic(s) discussed as “Complete” in the Care Plan – Nutrition Education.
- E. Follows requirements listed in policy when accepting a HRCP from an outside dietitian.
- F. Don’t withhold food benefits or limit to monthly issuance when the participant doesn’t complete a high risk nutrition education appointment.
 - 1. Make one attempt to reschedule the high risk nutrition education contact.
 - 2. Document the attempt to reschedule in the participant’s file.
 - a. Staff document in a consistent location in the participant’s file according to the clinic’s practice.

POLICY: Provide Drug and Other Harmful Substance Information

Staff trained in offering nutrition education must provide drug and other harmful substance information verbally to all participants, parent guardians, and caretakers at the initial certification.

Staff must provide this information verbally at a subsequent certification if the participant has a lapse in participation of 2 years or more.

PROCEDURE:

Staff:

- A. Verbally share that alcohol, cannabis, and drugs may be harmful to the health of the participant and their family as part of the nutrition education provided during the initial certification or at the subsequent certification when the participant has had a lapse in participation of 2 years or more.
 - 1. Select the Substance Use Information nutrition education topic in the participant's file on the Nutrition Education screen to document this requirement was met.
 - 2. Write a note in the Individual Care Plan as needed, for example when the participant needs additional follow-up and support.

Note: Staff are not qualified to diagnose drug and other substance abuse or expected to provide substance abuse counseling. Rather, they are to provide information that these substances can be harmful and provide referrals as appropriate.

Section 3: Remote Nutrition Education Contacts

POLICY: Remote Second Nutrition Education Contacts

Staff must offer participants the option to receive a remote second nutrition education contact for both high risk and non-high risk appointments.

Remote means providing the contact by telephone or video chat.

Staff must ensure the content of the remote nutrition education contact meets the requirements listed in the following policies:

- [Second Nutrition Education – Individual](#)
- [Second Nutrition Education – High Risk with a WIC Nutritionist](#)
- [Second Nutrition Education – Group Class](#)

PROCEDURE:

Staff:

- A. Offer the remote second nutrition education contact according to the policies referenced above.

Information:

Remote nutrition education contacts can be beneficial in the following situations:

- The participant prefers remote contacts due to transportation issues, childcare, etc.
- The participant hasn't received a nutrition education contact and staff can't schedule or reschedule one before the end of the participant's certification period.
- The staff wasn't available to complete the scheduled nutrition education contact for the participant due to clinic scheduling issues (e.g., staff illness, transportation problems because of severe weather conditions, or other clinic scheduling issues).
- The participant can't attend the nutrition education contact in person due to illness, imminent childbirth, inclement weather, work schedule, or a medical reason that prevents the participant from coming to the clinic.

POLICY: Online Nutrition Education Contacts

Staff must:

1. Offer online nutrition education as an option for the second nutrition education contact to participants any time during the certification period.
 - WIChealth is the only accepted platform to offer online nutrition education for Washington WIC.
 - Only offer WIChealth to participants who speak or read English or Spanish. WIChealth lessons are only available in these two languages at this time.
 - The WIChealth online lesson is relevant to the category, needs, and preferences of the participant or parent guardian.
 - Online nutrition education can't be used to meet the high-risk care plan requirement for participants designated as high risk. However, it could be offered as a second nutrition education contact after a high risk care plan is completed.
2. Document the intent to complete nutrition education using WIChealth in the participant's file.
 - Schedule a WIChealth lesson in the participant's file in the Nutrition Education Topics container on the Nutrition Education screen, **or**
 - Write participant's intent in their file in a consistent location according to clinic practices.
3. Assess if the participant did a WIChealth nutrition education lesson by confirming an online lesson is documented as "completed" on the Nutrition Education screen.
4. Not withhold food benefits or limit to monthly issuance when the participant doesn't complete a WIChealth online lesson or misses a nutrition education appointment.
 - Make one attempt to reschedule a nutrition education contact and document it in a consistent location in the participant's file.

PROCEDURE:

Clinic staff:

- A. Create a staff account and visit the website at www.wichealth.org and become familiar with its content.

Note: Staff send an email to wawictraining@doh.wa.gov to request WIChealth account.

- B. Offer the option of online nutrition education to English and Spanish-speaking or reading parent guardians or participants and determine their interest.
1. Frontline staff or certifiers can offer the WIChealth lesson to participants.
- C. Document that the participant intends to complete nutrition education using WIChealth in the participant's file.
1. Schedule a lesson in the participant's file as a topic on the Nutrition Education screen, or
 2. Write participant's intent in their file in a consistent location according to clinic's practice.
- D. Inform the person how to access WIC Health:
1. Provide the www.wichealth.org link or direct participant to the WIChealth button in the WICShopper app.
 2. Provide the Family ID, agency name, and clinic name.
 3. The person will create a username and password to set up their account.
 4. Encourage them to complete a lesson of interest for themselves or their child.
- E. Assess if the participant completed the nutrition education lesson on WIChealth at the time of or prior to the next benefit issuance.
1. Confirm in participant's file that the participant completed a lesson.
 - a. View the Nutrition Education screen in Cascades under Topic Status to see if WIChealth lesson was completed.

- b. The participant file in Cascades will be automatically updated to document the lesson within 24 hours after completion.
 - 2. Ask if they have any questions about the lesson.
 - a. Call, text or email the participant when the participant isn't in the clinic.
 - 3. Ask if they need any changes to their food benefits and issue benefits.
- F. Don't withhold food benefits or limit to monthly issuance when the participant doesn't complete an online nutrition lesson.
 - 1. Make one attempt to reschedule a nutrition education contact (e.g. try an online lesson again, individual nutrition education).
 - 2. Document the attempt to reschedule in the participant's file.
 - a. Staff document in a consistent location in the participant's file according to the clinic's practice.

Section 4: Refuse to Participate in Nutrition Education

POLICY: Refuse Nutrition Education

Staff must:

1. Offer nutrition education to participants or parent guardians at all certifications, mid-certification health assessment and nutrition education contacts.
2. Offer all WIC services, including food benefits, even when participants or parent guardians refuse to attend or participate in nutrition education.

Refusing nutrition education includes participants or parent guardians stating they don't want to:

- Receive any nutrition education during a certification, mid-certification health assessment, or a nutrition education contact.
 - Attend a second nutrition education contact of any type (for example individual, group, or high risk) or offered by any method (in person, remote, or online.)
3. Reassess the participant's interest in nutrition education at each certification and document if the person refuses.

Note: Refusing to participate in the nutrition education second contact should be rare. Using participant centered skills and offering a variety of flexible options that consider the interests of the participant or parent guardian helps ensure the education is positive, meets their needs, and supports behavior change goals.

PROCEDURE:

Staff:

- A. Determine if the participant or parent guardian is refusing to participate in nutrition education or if a different time or method would be better.
 1. Ask a question such as, "Would you like another day or time that's better for you?" or "Would you prefer to have a one-on-one appointment or complete an online nutrition lesson?"
 - a. If the person answers that a different day, time, or method would be better for them, staff schedule the nutrition education contact.

- b. If the person answers that they don't want any nutrition education of any type, staff document this as a refusal.
 - B. Document the refusal in the participant's file.
 - 1. Select Refuse on the Nutrition Education screen when the participant or parent guardian state they don't want to participate in nutrition education.
 - a. Select the appropriate Refusal Reason and add comments as needed.

Note: Staff can't select the Refuse button if a topic was selected on the same day.
 - 2. Place a note in the Family Alert that the participant or parent guardian refused nutrition education so other staff don't schedule this type of appointment.
 - a. Participants may "no-show" for a nutrition education appointment. A "no-show" isn't considered a refusal.
 - b. Staff don't need to reschedule participants who stated they refuse nutrition education.
 - 3. Document this information in the participant's Individual Care Plan.

Note: Offering nutrition education and documenting a participant's or parent guardian's refusal is required for each certification period.
 - C. Issue food benefits when participants or parent guardians refuse to attend or participate in nutrition education.
 - 1. Don't withhold food benefits or limit to monthly issuance when the participant refuses to participate in nutrition education.
 - E. Reassess the participant's interest in nutrition education at each certification and document in the person's file.

Information

Refusing to participate is not:

- The participant or parent guardian stating they're unable to stay for the second nutrition education contact because of an illness, family conflict, or other reason that conflicts with the scheduled nutrition education second contact appointment.
- The participant or parent guardian not showing for the second nutrition education contact appointment because of an illness, family conflict, or other reasons with the scheduled nutrition education second contact appointment.
- The participant or parent guardian has repeated no-shows or missed second nutrition education contacts for transportation issues, family conflicts, or other reasons that conflict with the scheduled nutrition education second contact appointment.

Section 5: Nutrition Education Materials

POLICY: Accessing WIC Nutrition Education Materials

Staff determine which WIC materials, if any, may be helpful to discuss or reinforce nutrition education with participants and parent guardians.

Staff access WIC nutrition education materials to offer participants and parent guardians, in the following ways:

- WIC Library on the WICShopper app.
- DOH WIC webpage: [Nutrition Education Resources for Families, Publications for WIC Participants](#).
- Printed nutrition education materials may be available from [myPrint](#) or can be printed from the web:
 - Consider print materials for when electronic access isn't an option or doesn't meet the participant needs.
 - Consider participants' language needs for written materials and order as appropriate.
 - Follow the order limit guidelines for the different materials.

Note: Staff tools, forms, WIC Cards, and participant WIC Card materials are also ordered from [myPrint](#). Order WIC Cards separately from other materials.

PROCEDURE:

Staff:

- A. Determine which WIC materials, if any, may be helpful to discuss or reinforce nutrition education with participants and parent guardians.
 1. Assess if person uses the WICShopper app and share how to access nutrition education material in the app.
 2. Determine if person has access to the internet and offer a link to nutrition education material on the web.

3. Offer a printed copy of the nutrition education material when the person is unable to use above options or print a copy from the web.
- B. Follow the instructions on the [WIC Program Forms and Materials – Ordering Materials](#) web page for printed materials.
1. It's recommended to use the staff email address for the log in ID.
- Note: Washington WIC translates materials into many languages but may not print all languages. See the [WIC Program Forms and Materials – Nutrition Education](#) page to access and print materials in other languages not available from myPrint.
- See [Volume 1, Chapter 15 – Breastfeeding](#) for information about ordering breastfeeding materials.
 - See [Volume 1, Chapter 22 – Issue WIC Food Benefits](#) for information about ordering WIC Cards.
- C. Follow order limit guidelines for each publication or material.
1. Order limits are by clinic. For staff ordering for multiple clinics, please enter in the Shipping Information field: the names of the clinics you're ordering for, (e.g., *"This order includes materials for 3 clinic sites: North, East, and West Clinics."*)
- Note:** Staff must order WIC Cards separately. Don't order WIC Cards along with other materials.
- D. Keep the confirmation email sent from [myPrint](#).
1. The confirmation email includes the approximate shipping delivery date, order number, and other information needed if there are questions about your order.
 2. Staff can delete the email after receiving the order.

Information:

Nutrition education materials provided for clinics in [myPrint](#) have been reviewed to include these (and other) important criteria: USDA audience tested messages, categorically appropriate, culturally appropriate, current nutrition risk messages, behavior change tips, positive health behaviors, multiple languages, emotion based messages, readability, etc.

POLICY: Evaluate Non-DOH Nutrition Education Materials

All nutrition education materials and resources provided to participants (i.e., not provided through DOH WIC) must be evaluated and meet the following criteria:

1. Be accurate and up to date.
2. Be from a reputable, evidence-based source.
3. Be culturally appropriate.
4. Support breastfeeding as the optimal method of infant feeding and be consistent with the WHO (World Health Organization) recommendations.
5. Be easy to read in terms of font, reading level and amount of white space.
 - a. Materials should be written at a 4th to 6th grade reading level.
6. Contain the non-discrimination statement when the material references the WIC program as follows:
 - Use the full version of the statement when the document discusses eligibility or outreach for the WIC program.
 - Use the short version of the statement when the full non-discrimination statement isn't required as described above.
 - The font size of either the full or short statement must be equal to the font size used in the rest of the document.

The WIC Nutritionist:

- Screens non-DOH nutrition education materials for content, appropriate reading level, cultural relevancy and participant centered approach when creating materials or obtaining them through outside sources.
- Assess all materials and resources to avoid product promotion.

PROCEDURE:

The WIC Nutritionist:

- A. Evaluates non-DOH nutrition education materials to ensure they meet criteria listed in policy above.